

Adult Disease Prevention Part 2 — CV, Psychosocial & Family/Safety

Average-risk adults. Adapted from TeachIM.org (CC BY-NC 4.0); USPSTF/ADA/ACIP + 2026 ACC/AHA verified. Educational use — clinician-reviewed.

Psychosocial (screen all, annually)

Domain	How
Alcohol	SASQ or AUDIT-C (CAGE = dependence, not screen)
Tobacco	Assess every visit (A); ask "ever" + all products
Drugs	Single-item: illegal/non-medical Rx use in past yr
Depression	All adults — PHQ-2/4/9
Anxiety	Adults <65 — GAD-7

NIAAA unhealthy use: M 21–64 >4/day or >14/wk; M ≥65 & all F >3/day or >7/wk.

Cardiovascular

Target	Who / how
HTN	Office BP all; annual if ≥40/at risk; confirm w/ home/ ABPM
Obesity	BMI all (B); ≥30 → behavioral referral
Diabetes	ADA: all at 35, or overweight/obese+RF any age; repeat q3y
Lipids	40–75 +RF, 10-yr risk >10% → statin (7.5–10% selective)
AAA	One-time US, men 65–75 ever-smoked (not women)

Activity: ≥150–300 min/wk moderate (or 75–150 vigorous) + strengthening ×2/wk.

2026 ACC/AHA dyslipidemia

Use **PREVENT** (30–79), not Pooled Cohort (overest. ~40–50%). Tiers: low <3% · border 3–<5% · interm 5–<10% · high ≥10%.

"Earlier & lower, for longer."

LDL targets: very-high <55, high <70, interm-on-statin <100. Earlier statin if FH, ≤30 w/ LDL ≥160, strong family ASCVD, or high 30-yr risk.

Family & Safety

Item	Who
Folic acid 0.4–0.8 mg	All who may become pregnant
Falls (3-Q screen)	≥65 / at risk, annually → exercise (STEADI)
Osteoporosis (DEXA)	Women ≥65; postmenopausal <65 + RF (men: insufficient evid.)
IPV (HITS/HARK)	Reproductive-age women, annually, private
Advance care planning	All ages (esp. comorbid/older): DPOA, living will, MOST/POLST

Vaccine note (2024)

PCV21 now an option for adults ≥50 (cases say PCV20 — still valid). RSV for ≥75 (and 60–74 high-risk) by shared decision.

Framework

5 domains: Cancer + ID (Part 1); **Psychosocial, Cardiovascular, Family/Safety** (here). Average-risk recs; family hx/comorbidity shifts toward diagnostic testing.